

Supporting Your Autistic Relative at Work

For family, partners & friends — supporting an autistic adult who is in employment.

The one idea

Autism, through **monotropism**, means your relative's attention locks into a few deep, intense "tunnels." In the right job that's a superpower — sustained concentration, reliability, pattern-detection, depth and honesty. In the wrong one (open-plan offices, constant interruption, office politics) it drives chronic depletion, masking and **autistic burnout**. Many working autistic adults — especially women — were identified only in adulthood. Your part is to believe them, back their accommodations, and help them read when the tank is empty.

What you may see — and what's really going on

You may see...	What's often happening
Exhausted, can't recover after work, losing skills	Autistic burnout — distinct from occupational burnout and from depression
"Fine" in public, collapses for the weekend	Heavy masking ; the cost is paid out of sight
Clashes with a manager; dreading the office	Often an environment-fit problem, not a competence problem
Won't see a doctor; ignores own pain or symptoms	Interoception differences — internal signals don't register when absorbed
Wants a quieter job, fewer hours, or a break	A legitimate, evidence-aligned recovery move, not "giving up"

Try this

- **Believe and validate** — late-diagnosed adults are often disbelieved by family; listening is transformative.
- **Back accommodations and rights** (Equality Act / ADA) — help them articulate what helps: quieter workspace, written instructions, fewer meetings, asynchronous communication.
- **Take burnout seriously** — if they're burning out, the evidence-backed direction is **reduce demands and expectations and allow unmasking**, not "push through."
- **Help with the practicals** if asked: navigating HR, occupational health, benefits and healthcare.
- **Respect autonomy** — support their choices, including reducing hours or changing role.

Avoid this

- Equating their public composure with being "fine," or dismissing exhaustion as laziness.
- Pressuring them to "just get on with people" or mask more.
- Making decisions *about* them *without* them.

When to get more support

Watch for **autistic burnout** and **suicidality** — burnout is linked to it. Address co-occurring **ADHD (AuDHD)**, **anxiety**, **depression**, **sleep** and **GI conditions** with autism-informed clinicians. Where burnout is severe, validate time off or a role change as recovery, not weakness.

If your relative is a woman

Women are over-represented among the “lost generation” of late-diagnosed adults and mask most heavily. Take a lifelong pattern of social/sensory difference and private exhaustion seriously, even with a “successful” exterior.

About this guide

*AI-assisted, derived from this project's research drafts — the **Family guide** (Part 4) and **Monotropism** brief, with the **Lifespan Practitioner & Health-Care guide** (Part 3.4). Uses identity-first language. **Informational — not medical advice or a diagnostic tool**. Fit it to the individual.*

Key sources. Murray, Lesser & Lawson (2005); Murray, F. (2023); Dwyer (2025); Raymaker et al. (2020); Mantzalas et al. (2022); Pearson & Rose (2021); Knight (2021). Full citations are in the source drafts.